PATIENT MEDICATION SCHEDULE

Post-Operative Recovery Plan | Following Appendectomy | Governed by Standard Clinical Prescribing Guidelines

MEDICAL NOTICE: This medication schedule has been prepared by the prescribing physician based on the patient's current condition. Do not alter dosage or stop any medication without consulting the prescribing physician. In case of severe side effects or emergency, contact the hospital immediately or call local emergency services.

Plan Date	15th September 2026
Plan Type	Short-Term Post-Operative Medication Plan (14 Days)

PRESCRIBING PHYSICIAN

Full Name	Dr. Bilal Ahmed, MBBS, FCPS (General Surgery)
Registration No.	PMC-52930
Clinic / Hospital	Liaquat National Hospital, National Stadium Road, Karachi
Phone	+92-21-34412500

PATIENT

Full Name	Mr. Hamza Sheikh
CNIC	12345-1234567-3
Date of Birth	5th June 1998 (Age 28)
Phone	+92-300-0000003
Medical Record No.	MRN-2026-93417

CLAUSE 1 — PROCEDURE & CLINICAL SUMMARY

Procedure Performed	Laparoscopic Appendectomy
Date of Surgery	14th September 2026
Post-Operative Day	Day 1 (discharged same evening)
Surgical Site	3 small laparoscopic incisions, lower abdomen
Known Allergies	Sulfa drugs (documented reaction: hives)
Weight / Height	76 kg / 5'10"

CLAUSE 2 — MEDICATION SCHEDULE (14-DAY COURSE)

The following medications have been prescribed for the post-operative recovery period. The full antibiotic course must be completed even if symptoms improve earlier, unless otherwise instructed by the Prescribing Physician.

Medicine Name	Dosage	Morning (8 AM)	Afternoon (2 PM)	Evening (8 PM)	Night (11 PM)	Duration
Cefuroxime (antibiotic)	500 mg	1 tablet	-	1 tablet	-	7 days (complete full course)
Metronidazole (antibiotic)	400 mg	1 tablet	1 tablet	1 tablet	-	7 days (complete full course)
Diclofenac (pain relief)	50 mg	1 tablet	-	1 tablet	-	5 days, then as needed
Omeprazole (stomach protection)	20 mg	1 capsule	-	-	-	10 days, before breakfast
Tramadol (breakthrough pain, as needed)	50 mg	As needed	As needed	As needed	As needed	Max 4 doses/day, first 3 days only

CLAUSE 3 — ADMINISTRATION INSTRUCTIONS

3.1 Antibiotic Compliance: The full 7-day antibiotic course must be completed in its entirety, even if the Patient feels fully recovered, to prevent incomplete treatment and antibiotic resistance.

3.2 Pain Management: Diclofenac should be taken on a scheduled basis for the first 5 days for consistent pain control, after which it may be taken only as needed. Tramadol is reserved for breakthrough pain only and should not be combined unnecessarily with Diclofenac.

3.3 Stomach Protection: Omeprazole must be taken 30 minutes before breakfast for optimal effectiveness in protecting the stomach lining from the anti-inflammatory medication.

3.4 Wound Care: Keep incision sites clean and dry; the Patient may shower after 48 hours but should avoid soaking the wound area (baths, swimming) for at least 10 days.

CLAUSE 4 — ACTIVITY & DIETARY INSTRUCTIONS

4.1 The Patient should avoid heavy lifting (more than 5 kg) and strenuous exercise for at least 4 weeks following surgery to allow proper internal healing.

4.2 Light walking is encouraged starting the day after surgery to reduce the risk of blood clots and support circulation.

4.3 A light, easily digestible diet is recommended for the first 3-4 days, gradually returning to a normal diet as tolerated, with adequate fiber intake to prevent constipation associated with pain medication.

4.4 Adequate hydration (at least 8 glasses of water daily) is recommended, particularly while on antibiotic and pain medication.

CLAUSE 5 — WARNINGS, SIDE EFFECTS & PRECAUTIONS

5.1 Common side effects may include mild nausea (antibiotics), drowsiness (Tramadol), and mild stomach discomfort (Diclofenac if taken without food).

5.2 Signs requiring immediate medical attention include: fever above 101 degrees Fahrenheit, increasing redness or discharge from incision sites, severe abdominal pain, or persistent vomiting.

5.3 Given the documented Sulfa drug allergy, the Patient must avoid any Sulfa-based medications and inform all treating providers of this allergy.

5.4 The Patient should not drive or operate machinery while taking Tramadol due to its sedative effects.

CLAUSE 6 — FOLLOW-UP & MONITORING SCHEDULE

Wound Check / Suture Review	21st September 2026, 10:00 AM
Final Post-Operative Review	5th October 2026, 10:00 AM
Return to Normal Activity	Expected after 4 weeks, pending final review

CLAUSE 7 — EMERGENCY CONTACT INFORMATION

Clinic Emergency Line	+92-21-111-456-456
Nearest Emergency Room	Liaquat National Hospital Emergency, National Stadium Road, Karachi
Family Emergency Contact	Mrs. Sara Sheikh (Mother) — +92-300-0000013

PRESCRIBING PHYSICIAN

Dr. Bilal Ahmed, MBBS, FCPS (General Surgery)

Signature: _____

Date: _____

PATIENT

Mr. Hamza Sheikh

Signature: _____

Date: _____